

SECTION II

The IFS Technique: Working with Protective Parts

CHAPTER 5

Getting Started and the Steps Involved

STARTING AN IFS SESSION

I frequently get asked questions like: “Do you do any psychoeducation about the IFS model before you start with a client?” and “How do you start a typical session?” I usually don’t do any psychoeducation before I begin working with a client, and I typically initiate a session as most therapists do. I ask open-ended therapist questions, such as: “What brings you in today?” or “How can I be helpful?” I’m typically curious to get to know the general landscape of parts, I’m interested in mapping out the various systems that emerge, and I’m hoping to get a list of wounds and determine an order of healing as exiles slowly reveal themselves.

Initially, clients will launch into a series of complaints. I’ll hear comments like: “I’m so glad I was able to get an appointment. I’ve really been struggling with my kids lately. They’re at a challenging age right now, and I’m not handling things very well. My wife and I aren’t on the same page at all about how to handle them, so we’re fighting a lot more lately. And to top it all off, my mother just broke her hip, and I’m super stressed about taking care of her with everything else I have on my plate right now.”

I may reply with: “Well, that all makes a lot of sense to me. Let me see if I have this right: One part of you isn’t handling things with your kids the way you wish, one part of you is fighting more with your wife lately, and another part of you worries about taking care of your mother. Is that right?”

“Yes, exactly,” I’ll often hear the client say.

My recommended approach is to listen to the client’s range of complaints. Then I repeat these complaints back to them, using their language and making sure they feel heard and validated. I also attach the word “part” to each of their struggles. (“One part of you is struggling with parenting duties.”) My rationale here is that I’m introducing the idea that they have different parts. Clients typically don’t take offense to this approach.

Next, I ask: “Which issue would you like to focus on first?” or “Which part is the most distressing to you right now?” Here I’m searching for the target part and will often suggest that the person *go inside and check*. “Going inside” is another new concept for them to consider.

This often elicits a response like: “What do you mean by ‘go inside’?”

I respond with: “You know—what does your gut tell you?” or “What is your sense about where we should start?” Most people know what this means and will begin to focus their attention internally.

Using this approach, we have successfully initiated IFS therapy—with no psychoeducation about IFS or how it works.

THE 6 F’S

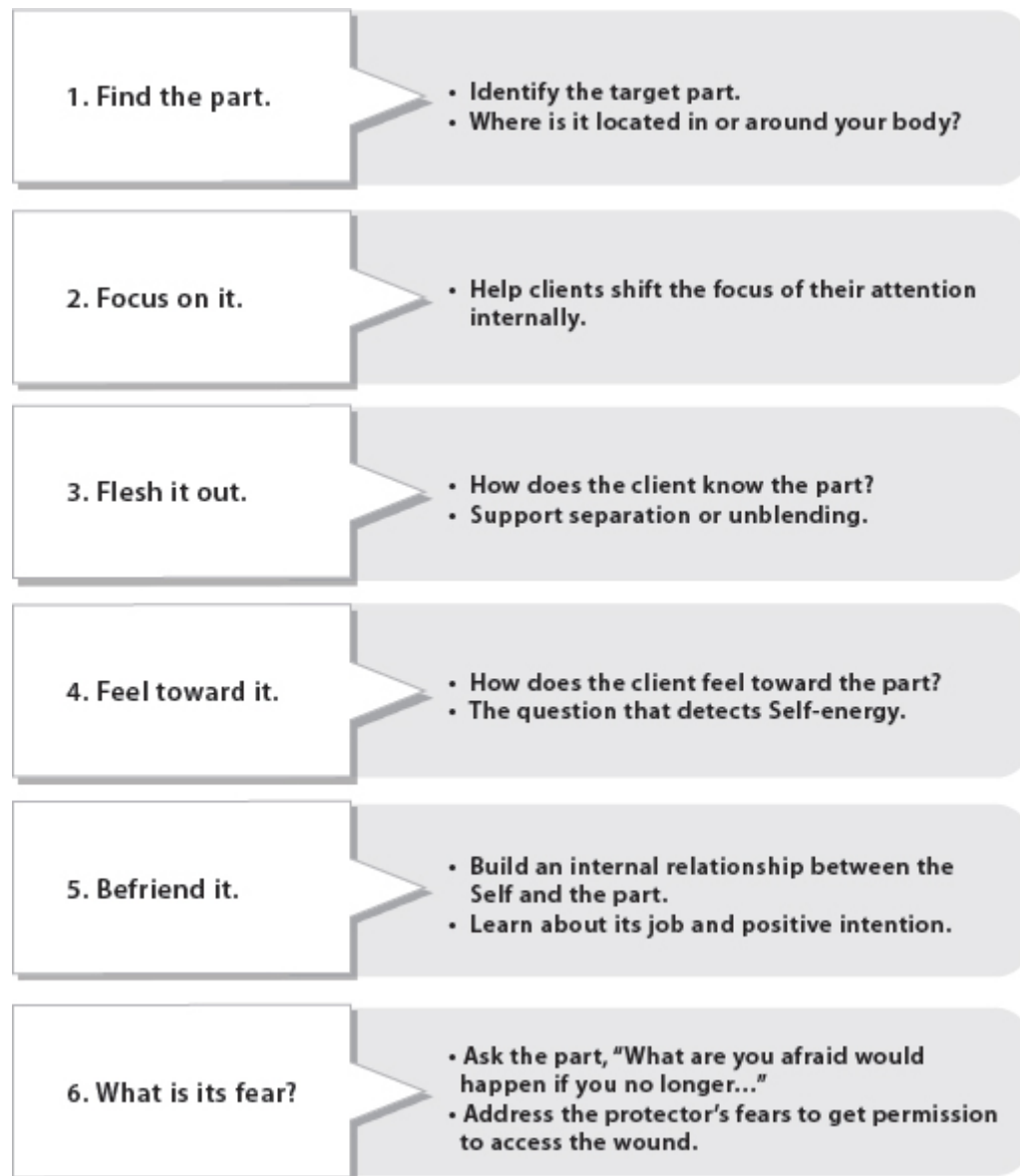
Once you and your client have identified a target part, this begins the sometimes complicated journey of gaining permission to access the client’s wound. This tends to be easier said than done, especially with clients who have extreme histories of trauma and neglect. And this is where therapists can easily get frustrated, lose confidence, and resort back to their old ways of conducting therapy.

It’s important to remember that the overarching goal of protective parts is to keep the hurt away. Protective parts will go to any length to distract, derail, confuse, and avoid moving toward the pain that an exiled part carries. Don’t take it personally when a client goes against your suggestion or wishes. Try to stay the course, and remember that protective parts may feel like you’re going against their purpose and agenda: to keep the pain away.

When encountering a range of protective parts, our goal is to identify which one the client’s system chooses to target, knowing that all protective parts have a wound beneath them they are shielding. We then help the client get to know the part independent of itself, acclimate with the system to which it’s connected, and facilitate the part separating or unblending from the Self.

Once we establish an internal relationship, it’s important to foster a trusting relationship between the client’s Self and this part. Harnessing the healing capacity of Self-energy, we have our clients share their love, clarity, and compassion for that part whenever it surfaces during a session. After establishing a safe relationship, we help the client learn about the part’s job and the fear associated with it, knowing that the underlying fear often connects to the wound in important ways. Once the fear and wound are determined, we provide an alternative solution to their situation by offering to help heal the wound they are protecting. *Your genuine attempt to listen to and understand the positive intention of protective parts will go a long way toward convincing them to permit you to access the wounded part.* It doesn’t matter which protective part you start with, as they all lead to wounds.

Listed here is an important chart that includes what we call the 6 F’s and the steps involved in identifying and working with protective parts, which I just generally described. Use this chart as a template to help you remember the steps when you’re working with clients. You can also refer to the *IFS Skills Training Manual* (Anderson et al., 2017) to learn greater details about the basics of the IFS model. It contains step-by-step instructions along with easy-to-use exercises and case examples.



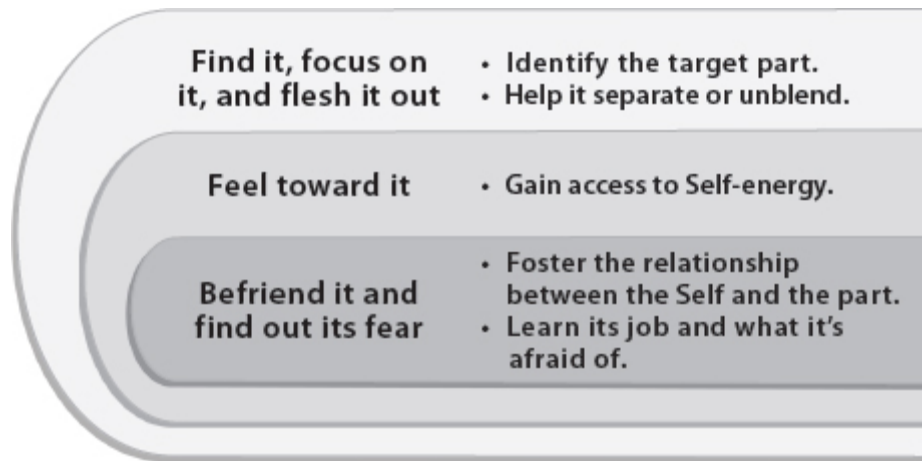
EXPANDING THE 6 F'S FOR TRAUMA

One of the challenges for IFS therapists is how difficult it can be to negotiate with protective parts and gain their approval to approach the exiled part. Trauma protectors are tricky, tenacious, and downright dangerous at times. It requires skills beyond the basics to effectively maneuver within the labyrinth of protective parts associated with trauma.

I generally like to break down the 6 F's into three different components. First, *identify a target part and achieve separation or unblending* (i.e., the first three steps of the 6 F's). Second, once that's successful, focus on helping the client *access Self-energy*—no easy task when it comes to complex PTSD, neglect, and DID—which we will expand on later in this book. Third, *build the internal relationship and determine the part's job and fear*. I find breaking down the 6 F's into these three different components prevents you from getting lost, derailed, or confused by the process. Remember

that protective parts don't want you to go to those painful places, and they'll do just about anything to distract you.

The following is a chart that breaks down the 6 F's into these three components and explains the main function of each component.



Once you get familiar and comfortable with the 6 F's, it usually requires additional expertise beyond the 6 F's to gain permission from protective parts to access the wound, especially when you're engaging parts associated with complex trauma. I call these skills *Beyond the 6 F's*, which we'll learn more about in [chapter 9](#).

CHAPTER 6

The First Three Steps: Identify the Target Part and Help It Separate

IDENTIFYING THE TARGET: VALIDATE FIRST

As stated previously, the first three steps of the 6 F's—find, focus, and flesh out—are used to identify the target part and to help it begin to separate or unblend. This is often more difficult than expected, especially in complex PTSD. Because of the intensity of trauma, many clients spend a great deal of time *within* their parts, moving from one crisis and one protective part to another, with very little access to Self-energy.

Identifying the target part can be tricky and frustrating to achieve, especially when someone is frequently triggered and often within their experience. When clients are blended, it can be incredibly important and often challenging to validate and acknowledge what they're experiencing in the present moment, particularly if it's largely a distorted version of what's happening in reality. Remember, there's often a sliver of truth to any triggered situation. Validating the part's experience first goes a long way toward gaining permission in the end.

I personally find it challenging to validate my client's parts, especially when they are triggered and their traumatized parts have a distorted view about me. For example, I've heard clients say: "I saw the way you looked away just then. You're not interested in taking me on as a client. Most of my old therapists have called me a borderline. I'll bet you agree."

When my parts are not activated and I'm in Self-energy, I can reply as such: "It sounds like there's a part of you that's worried I won't like you or be willing to work with you. Is that correct?" But when the distortion activates my parts, I've been known to respond with: "Actually, that's not how I'm feeling. I know myself pretty well, and I'm happy to tell you what I'm feeling if you want me to."

Correcting the distortion will further inflame the client's parts, while providing validation first is usually more proficient and effective, and it allows their parts to soften and their Self to more easily emerge.

Identifying the target part involves knowing, seeing, and validating it, even if its perspective is largely from another time and place. Identifying a target part is the first step toward unblending. By

recognizing, validating, and naming the target part, the client is less likely to remain blended in their experience. This opens the door for further exploration and separation, and it enables the target part to more easily connect with the Self. The key for us as therapists is to avoid getting triggered by the distortion, no matter how outrageous it may seem to you in the moment.

MIND AND BRAIN

Research from the field of neuroscience can help provide an understanding of how different parts express themselves. This provides therapists with a greater ability to sit with, identify, and validate the trauma distortions that are commonly associated with triggered parts. I love how Dan Siegel's work in particular helps us understand the difference between the mind and the brain, which has several implications when it comes to IFS. He describes the mind as an embodied and relational, self-organizing emergent process that regulates the flow of energy and information both within and between (Siegel, 2017). He also says the mind deals with energy and the prefrontal cortex (PFC), the anterior portion of the brain that integrates emotion and cognition, helps direct the flow of energy.

As therapists, I believe that we work primarily with the mind and that the mind can affect and change the brain. Consider that when we ask clients to focus on their heart or when we hear them describe the fluttery feeling their anxious part is creating in their stomach, their PFC makes this possible. The mind is capable of shifting or changing states, and we rely on this in IFS when we ask a part to step back and separate or to shift from one state (or part) to another. The imagination is also in the mind and is known to be a powerful agent that promotes neuroplasticity (Doidge, 2007).

Norman Doidge and others first helped us understand that the brain can change via its capacity for *neuroplasticity*. When I went to medical school in the mid-1980s, this information was not yet known. Each time we have an experience, neurons fire in the brain, and this firing strengthens and grows new neural synapses (Doidge, 2007). We also know that we're capable of growing new nerve cells through a process known as *neurogenesis*. When the nervous system is working smoothly or harmoniously, Dan Siegel calls this *neural integration* and says it's the basis of healthy self-regulation (Siegel, 2017). This is what I like to refer to as mental health.

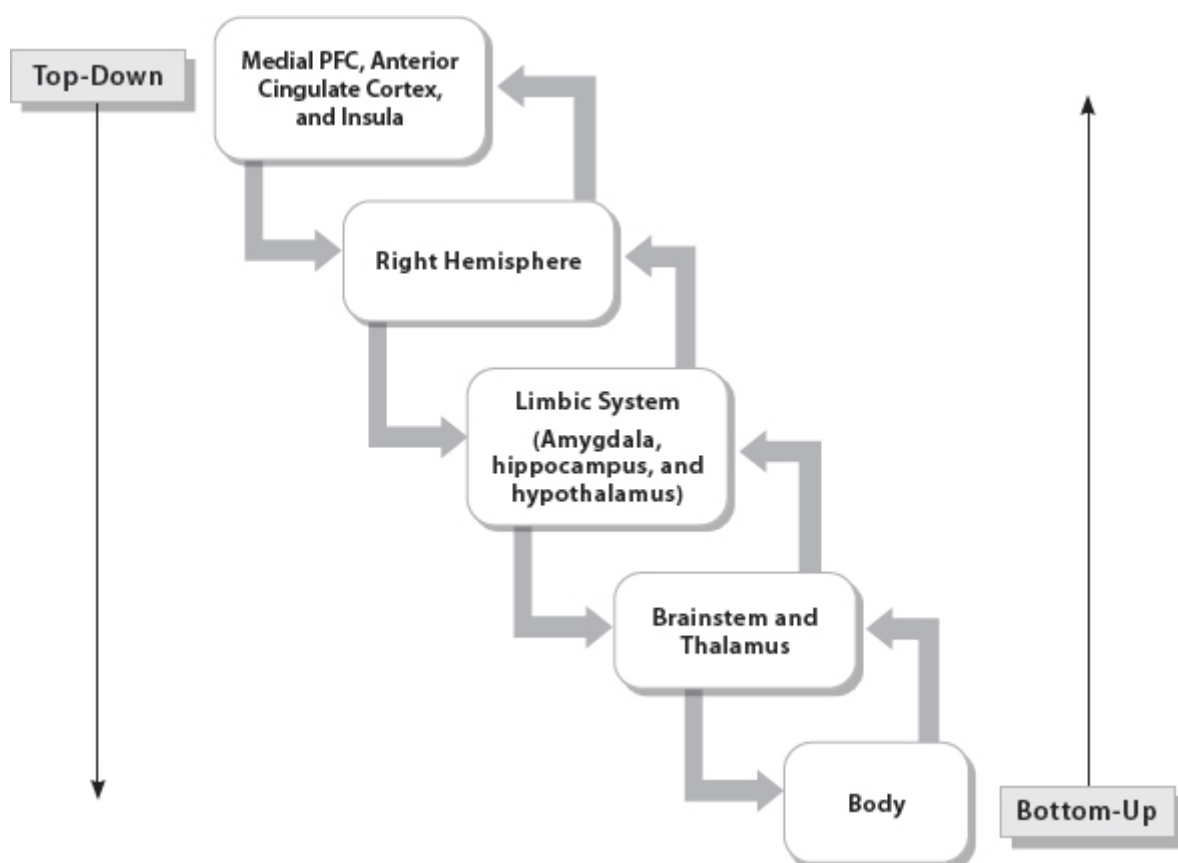
As the mind is related to function, the brain is related to structure. Different from the imagination and the mind, the brain consists of nerve cells, dendrites (the projections off the nerve cells), synapses (the space between nerve cells), and neurotransmitters (the chemicals that help nerve cells communicate with each other). It's also important to remember that the nervous system is not only located in the brain; it's everywhere throughout the body.

The brain alone has over 85 billion neurons, 100 trillion synapses, and 100 chemical neural transmitters. To put this in perspective, psychiatric medications primarily work on five of these chemical neural transmitters: dopamine, norepinephrine, serotonin, glutamate, and gamma-aminobutyric acid (GABA).

NEURONS TO NETWORKS TO PARTS

We know that a series of neurons string together to form what is called a *neural network*. Neural networks communicate with one another throughout the brain to give rise to thoughts, feelings, physical sensations, and complex actions.

One particular network that is relevant for clients with trauma histories is the vertical network or, as some refer to it, the top-down/bottom-up network. This network takes input from the body and brings it up into the brain, where it integrates emotional, cognitive, and physical information. Next, it travels to the PFC, which integrates and processes the information before it determines the appropriate response. This information is then brought back down to the body. Here is a diagram of the vertical network. We will revisit this network when we discuss the neurobiology of PTSD.



Trauma networks can be powerful and potent, holding very strong views, perceptions, and emotions. Louis Cozolino says that the vast majority of the input to the cortex comes from what is

already inside our brain. We scan for examples that prove our preexisting beliefs, which are primarily driven by fear to avoid danger (Cozolino, 2010). This speaks to the experience I have when I'm working with clients who are in their trauma and they interpret what is happening in the present through the lens of their past. In contrast to "seeing things through rose-colored glasses," it's more like "seeing the world through trauma-colored glasses."

As mentioned earlier, it is beneficial for therapists to acknowledge and validate this trauma distortion, which increases the likelihood of it softening. I often use the following phrase from Brené Brown when talking to clients who are working through past experiences: "What's the story you're telling yourself right now?" This is a non-shaming way to ask about their parts' perceptions of the past that are currently being projected onto the present situation. These stories or internalized beliefs and feelings are potent, tenacious, and overpowering and have a formidable influence over our clients and their often-distorted view of the world.

A few years back, I experienced the power of distortion from my younger parts and the influence they had had on my present-day life. At the time, my father-in-law was sick, and my husband asked if he could visit him alone to spend some one-on-one quality time with him. This certainly seemed like a reasonable request to me, so I said of course. While my husband was away, I learned that the trip had morphed into a mini-family reunion. I began to experience, as they say, FOMO (fear of missing out).

As the days progressed, I also became aware of my younger parts and their feelings of being alone, abandoned, and increasingly desperate for connection. At the same time, I was aware of an angry part of me that was escalating. I began distancing myself from my partner in the service of protecting my younger parts from feeling more distressed and alone. It was utterly strange to be able to hold the present-day perspective, as well as feel both angry, hurt, and abandoned in connection to my past. I believe I was fortunate to have enough Self-energy present to witness all of this unfold within myself, which was a new experience for me.

I recognized how my angry part felt compelled to disconnect from my husband, and I identified how this angry part was trying to help my younger self. I also clearly saw how my angry part was perpetuating the problem. The angrier it got, the more it disconnected me from my husband; consequently, the lonelier and more abandoned my younger part felt.

My Self took charge and was able to reach out to my husband and be vulnerable with him. I asked my husband for help, who responded lovingly and caringly. Being able to experience all these

emotions and be present in these feelings, both past and present, allowed me to subsequently heal my young attachment wound in the course of my own therapy, which has had an enormously positive impact on my life.

Being able to experience the desperation of the wounded part and the tenacity of the protective part, while simultaneously witnessing how they unintentionally worsened the problem, was enormously valuable for me on a personal and professional level. I have a new level of appreciation for the power of traumatic distortion and the IFS phrase “protectors draw to themselves the very thing they’re protecting.” I experienced this dynamic firsthand, and now I’m better able to stay present with myself and with my clients when we’re blended, possessing a new level of appreciation as a result of this experience.

Protective parts come in a range of intensities and roles. What they all have in common is the desire and intention to keep the pain away in the service of self-preservation. *I speculate that parts live in the mind and that they utilize neural networks in the brain to express themselves.* I believe that protective parts, in particular, utilize unintegrated or dysregulated neural networks in the brain to express themselves. The parts that are not involved in protection or don’t carry wounds utilize integrated neural networks in the brain to express themselves.

GOING INSIDE

The process of *going inside* is not an easy one for many trauma survivors. First, most clients are used to traditional talk therapy, which primarily engages a storytelling part of the client. They need to adjust and become comfortable with focusing their attention internally versus externally. Second, the practice of going inside connects clients to their feelings and body sensations, which can be extremely uncomfortable for them because that’s often where traumatic experiences are held.

In my experience, the transition of going inside is more difficult for clients than accepting the notion that they have parts. Most people spend much of their time getting away from their Self and inner experiences. Here, we’re asking them to move toward their parts—to connect and listen to what’s going on inside without necessarily talking or thinking about it. We’re encouraging them to slow things down, turn inward, and get curious about what’s going on inside.

Some protective parts that are vague, distracting, disconnected, and avoidant of the pain will naturally emerge because focusing intently on the problem makes them nervous. Other protective parts will take over, flood the system, and overwhelm the client with excessive emotion in an unsuccessful attempt to be heard and helped. I’ve noticed that with clients who tend to become overwhelmed, I can spend an inordinate amount of time helping them try to achieve *a healthy*

distance and perspective from their feelings. However, with other clients—especially those who are numb and cut off—I frequently suggest they go inside and try to *move closer to and connect with* what they’re feeling.

Both types of clients, those who feel too much and those who feel too little, have difficulty making decisions for themselves because they’re disconnected from their true Self and living life predominantly through their parts. Clients with DID can rapidly switch from one side to the other within seconds, being overwhelmed in one moment and not feeling much at all in the next.

When you participate in traditional talk therapy, you and your clients are primarily engaged in what’s called *exteroceptive awareness*. You’re both focusing your attention externally and primarily engaging your PFC (Seppala, 2012). When you ask your clients to go inside, you’re requesting that they shift their attention internally and engage in what’s called *interoceptive awareness*. This relies on the insula (which integrates body awareness and emotion) and posterior cingulate gyrus (which connects to awareness, memory, emotions, and the default mode network). Both structures link to the limbic system and brainstem (Seppala, 2012), which are connected to the aforementioned vertical (or trauma) network.

In other words, when we ask our clients to focus internally, we are having them focus on the actual neural networks affected by their trauma; when we engage with them in talk therapy, we’re primarily having them engage the PFC. I’m not saying that you can’t heal complex trauma with talk therapy. However, I think it might take longer since clients cannot directly access the structures affected by trauma like they can when they are interoceptively connected. Consider also that research shows internal attention has a greater influence over our sense of happiness (Seppala, 2012).

IFS utilizes meditative practices primarily to help clients access Self-energy and aid in the unblending process. Meditation should not be compared to psychotherapy, as it is not a therapy modality. But clients who meditate can spontaneously experience moments of healing while engaged in meditation. There are many different forms of meditation, of course. Some forms of meditation suggest moving away from thoughts, feelings, and actions, while others encourage connecting with thoughts, feelings, and actions that show up with loving kindness or what is referred to as “metta.” The latter approach is what we encourage with our clients. Jon Kabat-Zinn espouses the notion that mindfulness meditation can help with the uncoupling of thoughts and symptoms from oneself (Kabat-Zinn, 2003), which is very much in line with the IFS perspective of unblending.

As I see it, we need to encourage clients to clear their minds, help their parts relax, and see if Self-energy will naturally show up. This is a way to slow down their systems enough to allow for the emergence of the Self. The goal here is to foster a trusting relationship between protective parts and

the Self. For example, I will frequently say: “What does this anxious part of you need right now to be able to trust your Self a little more?” This approach uses mindfulness to foster separation, help the Self emerge, and develop a trusting relationship between parts and the Self.

Britta Hölzel and colleagues have researched the brain regions affected by mindfulness meditation and have demonstrated that meditation decreases activation in the amygdala (assigns emotional significance to events) and increases activation in the PFC (integrates emotion and cognition), insula (incorporates body awareness), anterior and posterior cingulate cortex (impacts attention, memory, emotion, and self-identity), and temporal-parietal junction (also connected to self-identity; Hölzel et al., 2010, 2011). Many of these brain changes associated with mindfulness meditation are corrective experiences that help reverse the impact of trauma. When clients focus their mental energy internally, they are more likely to connect to the brain structures that were affected by trauma and need healing.

How clients and therapists focus their energy can have a powerful effect on the healing process. I’m often amazed at how much longer it takes my clients to unburden their wounds during individual therapy sessions in my office compared to how much more quickly this can happen during a training demonstration—when dozens of attendees engage in sending healing energy toward a chosen volunteer. It’s incredible! The cumulative effect of focused energy produced by a group of people toward an individual is extremely powerful.

Here’s the bottom line: Meditation is a useful tool to help clients unblend and access Self-energy. It positively affects the structures in the brain most affected by trauma.

FLESH OUT AND SEPARATE

Identifying parts and achieving separation (unblending) are among the main goals of the first three steps of the 6 F’s. However, these tasks are easier said than done with complex trauma survivors and can take many months to achieve in some cases. Openended inquisitive questions can help clients establish a relationship with the part instead of being blended “in” the part. Good examples include: “Does the part have a color associated with it?” or “Where is it located in your body?” or “Does it move around or have a shape associated with it?” or “Do you see it, hear it, or feel it?” If they can answer the question, it shows that they’re not totally blended with the part.

Early on in treatment, many trauma clients are primarily *in* their parts most of the time, so having them relax or separate, even slightly, can be challenging. With severely traumatized clients, it’s easy for therapists to get frustrated by the lack of progress. Often, parts don’t know or trust the Self enough to separate from it. In addition, “Self-like” parts will frequently show up and try to move the process along. You may hear the client compliantly say things like: “Sure, the part said it’s willing to step

back” or “Yes, I feel compassionate toward it.” This can be confusing because it sounds as if things are moving forward, yet no real progress or healing ever happens. When this occurs, therapists can get discouraged, give up on IFS altogether, and go back to their old way of working with clients.

These three initial unblending steps can be taxing. But I encourage therapists to be patient and make the necessary effort up front to gain real separation before proceeding with steps four, five, and six. It will save time down the road if the target part is truly willing to separate and genuinely grant permission to proceed.

Strangely enough, one of the ways to achieve separation is to briefly encourage blending. If your client is not on the severe end of the dissociative spectrum (as is the case with DID) and you encourage the part to take over and share what it’s feeling with you, as therapist, then the client’s Self will likely be listening in the background, hear what the part has to say, and often appreciate the role it’s playing.

In my personal therapy, I learned that my system generally blends first before any separation is possible. My parts tend to quickly take over and need to be heard before they are willing to relax and connect with my Self. Initially, I would resist this because it was so overwhelming for certain parts of me when intense parts took over. But over time, I’ve learned that this is the best way for my parts to truly achieve separation. They needed to blend first and say their piece; then they are willing to soften.

As I look back on this now, I sense that many of my parts were on their own for so long that they didn’t believe anyone could or would be there for them. I’ve become more comfortable with their need to express themselves in this way. My parts take over, say what they need to say, look up at me, and see that my Self is listening to them. Once this happens, they’re willing to relax. Mine is a “being in/before being with” or “take over and talk first, then separate later” kind of system. Remember, each system or collection of parts is different, and parts are capable of separating when they’re ready. It’s our job as therapists to be open and flexible and to learn how each system within our clients operates.

CHAPTER 7

Step 4: Self-Energy and Gaining Access to Internal Wisdom

Once the target part and other parts of the system have been identified and are willing to separate (the primary goal of the first three steps), the fourth step is to help clients gain access to Self-energy. We inquire about the presence of the Self when we ask questions like: “How do you feel toward the part that is depressed?” We then aim for responses aligned with what embodies Self-energy, such as: “I’m finally open and really ready to listen,” “I care deeply and want to help that part,” or “I’m feeling a great deal of love and appreciation for the depressed part right now.” We are looking for statements that exemplify the 8 C’s of Self-energy: *curiosity, clarity, confidence, calm, compassion, courage, creativity, and connectedness*.

I appreciate the perspective of Gary Zukav, someone Oprah Winfrey admires, who authored *The Seat of the Soul*. Zukav believes that the soul comes from a higher-order logic that is meaningfully reflective and connects to our emotions and heart (Zukav, 2014). He states that *incomplete* parts of the soul make up our five-sensory personality, which connects to our intellect and understanding and originates in the mind. He also believes that every discrepancy between our conscious mind and emotions points directly to a splintered aspect of the soul that requires healing. I believe this is consistent with the IFS perspective—that Self = soul = heart = emotion and parts = personality = intellect = conscious mind—and that healing is required when these components are fractured or out of alignment. To become whole, Zukav says, our soul must balance its energy.

I experience the Self in three different ways. First, I feel a tingling or flow of energy throughout my whole body, with a particular concentration in my arms and legs. I believe this happens when I connect to the energy in nature or my surroundings. Second, I have what feels like a titanium core inside of me that is solid and unwavering. When connected to this space, I feel calm and powerful; no one can push my boundaries or persuade me from my center. And when I speak from this place, people (my children, in particular) listen. The third dimension of Self-energy for me is when I connect to that which is beyond us: the spiritual realm. When I’m in the spiritual realm, I feel that all is and will be okay—that I can let go and things will work out exactly as they’re supposed to. I can let go and fully trust the greater good.

I encourage you to explore the different ways you experience Self-energy. Do you feel it in connection to your environment or nature? What do you feel in your body when you're in Self-energy? Do you connect with something spiritual and, if so, how do you experience that?"

COMPARING COMPASSION AND EMPATHY

As therapists, we know that there are different ways to *be with* our clients. At times, we can be fully engaged and present, and at other times we can be bored, distracted, or preoccupied. We can also feel triggered, totally overwhelmed, or overinvolved. Tania Singer, the head of the social neuroscience lab at the Max Planck Institute in Germany, and her group have been able to map out two important ways we can be with someone else. One is by having compassion; the other is by being empathetic. Singer defines compassion as a feeling of concern for the suffering of others, coupled with the motivation to help them (Singer & Klimecki, 2014). Neuroimaging studies show that compassion involves the care-seeking neural network, specifically the ventral striatum, pregenual anterior cingulate cortex, and medial orbitofrontal cortex. Empathy, on the other hand, is defined as resonating with others' suffering. It involves interoceptive awareness and consists of a neural network in the anterior insula and anterior midcingulate cortex. Singer's research shows that compassion is a sustainable state, although we can burn out from being too empathic for too long.

From the IFS perspective, I see compassion as an unblended state of "being with," which is more aligned with Self-energy. Empathy, on the other hand, is a blended state of "being in" our parts and is more aligned with protective parts and exiles. With compassion, we are there for the other person with some amount of distance, perspective, and a genuine desire to help them, which can be incredibly important and necessary, especially when a trauma survivor is overwhelmed and activated in the moment.

Compassion is useful for clients, as well as therapists, particularly during intense therapeutic moments, because it provides a sense of safety for all. I believe this aligns with the experience of Self-energy, which involves having presence and perspective while being open and available to help others. Parts of our clients will need and want this from us at times. *The energy of compassion is a positive force; it fosters trust and is, I believe, an antidote to fear.*

At other times, our clients or their parts will want us to truly "get" what they are feeling. Think of how many times you have heard: "How can you possibly help me? You have no idea what it's like to grow up with an alcoholic father and a verbally abusive mother?" or "You're so put together; your life is perfect." Here the client is saying: "I don't feel you empathically resonating with me right now." When we are able to resonate with someone else and with what they're feeling in the moment,

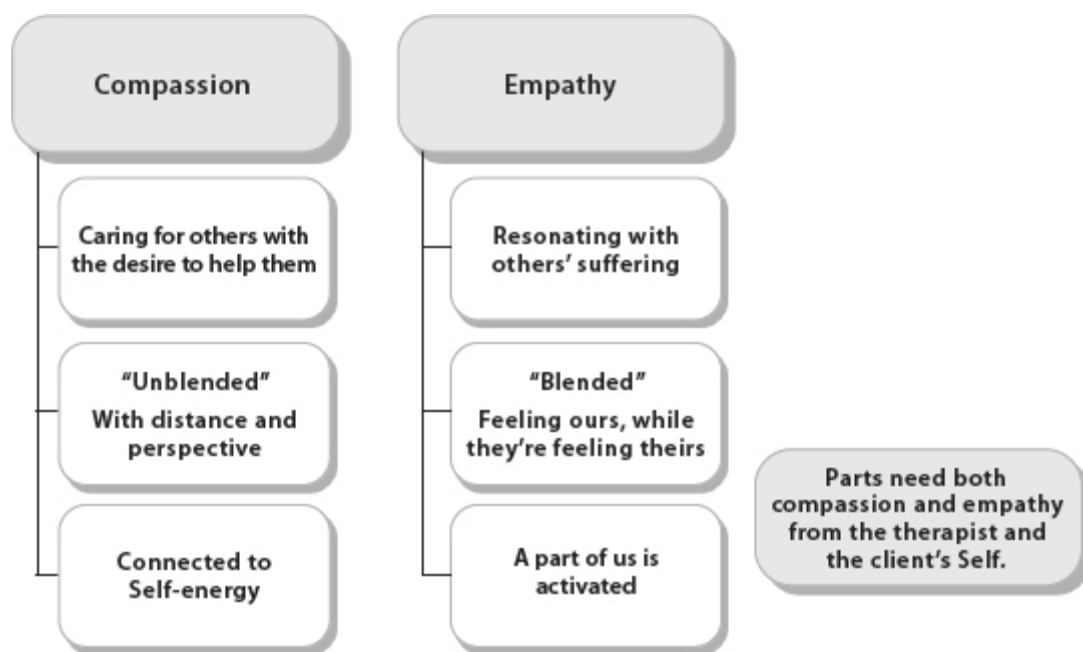
I believe we're *not* actually feeling what they are experiencing at all. Instead, we're connected to and feeling *our own experience* while they're undergoing their experience. With empathy, our protectors or our wounds can get activated when we're working with a client who is "in" their experience or blended with their parts.

For example, I recently welled up with emotion when working with a client who was overwhelmed and vulnerable upon talking about the difficulty of parenting children when they're totally out of control. I relived my own intense experiences while she relived hers. With empathy, our parts get activated, and we blend with them as we resonate with our client's pain. We don't necessarily need to have the same experience as our client, but we need to relate to the *feelings* associated with their event. Empathy can help us connect with and relate to our clients. We have something in common and identify with their anguish as we connect with our own similar past experiences.

With compassion, our parts are not activated and we are not blended. Rather, we remain in a calm state, connected with the client's presence and capable of identifying their pain without getting overwhelmed by it. *Truly being present with our clients requires a delicate balance of being empathically resonant (being with our own parts) while not getting too overwhelmed by them, as well as having a compassionate distance and perspective without being too withdrawn or detached.*

My experience is that parts of our clients need and desire both empathy and compassion from the therapist at various times. These parts also need and require empathy and compassion from their Self too. (More on this later, when we explore healing wounds in [chapter 23](#).) Empathy, when not overwhelming, can inform compassion. If I feel a bit of empathy, it can help me be present and emotionally engaged with my client while adopting a compassionate stance.

Too much empathy, however, can engulf us and block our ability to have the necessary distance and perspective to hold a compassionate position with our clients. Jen Kleiner, a friend and colleague of mine, reminded me that there are times when we can perceive the feelings of another person without necessarily feeling compassion for or empathizing with them. I agree with her; it is possible to sense another person's energy and emotions without activating our own feelings. This is what occurs with psychics and energy healers. They are trained to pick up and sense the feelings, thoughts, and beliefs in others.



Many therapists, myself included, are what the general public refers to as an “empath.” I grew up with people constantly saying to me: “You’re too sensitive” and “You react so strongly to everything.” I suspect these qualities identified by others relate to my natural temperament, as well as to my personal trauma history. It was common for me (although less so now after all the healing I’ve achieved) to be overwhelmed and drained after a full day of sitting with clients. I tended to take in and absorb what I heard, bringing these experiences and emotions home with me at the end of the day.

I suspect that many of us in the field are also “highly sensitive” people and magnets for other people’s feelings. Being sensitive can certainly be a gift as well as a curse. Fortunately, I have gotten much better at clearing the energy I’ve experienced with one client before engaging with the next. This is a big reason why I’m a strong proponent of the 50-minute therapy session: Having at least 10 minutes between sessions allows me to wipe the slate clean and reenergize before welcoming the next client.

Empathy as a Part

I’ll never forget the time my son was in the hospital after sustaining an injury from a mountain-biking accident. Visiting hours were coming to a close, and I wanted to stay with him overnight. I couldn’t bear the idea of him being all alone in that hospital room. “Papa, you can go home—I’ll be fine,” he told me.

As I drove home, Dick Schwartz called me to check in and see how my son and I were doing. I broke out into tears, feeling completely overwhelmed by the events of the day as I tried to explain to him what was going on.

“Pull over. Let’s see what’s going on with your parts,” Dick said.

My response was gruff and impatient: “Explore what’s going on? Didn’t you hear me? My son is in the hospital.”

Dick’s next few words took me by surprise. “I know, Frank, but let’s see if the part of you that’s so overwhelmed right now can separate and tell us more about what it’s holding.”

I parked on the side of the road, and we began to explore the overwhelmed part of me. Much to my surprise, I discovered that there was a wounded part of me overreacting to the thought of my son being alone and in pain. This part was rooted in me as a little boy, at a time when I was totally alone after being physically abused as a child. I recognized that this part had taken over and blocked my ability to be open, available, and loving toward my son. In retrospect, I think my son sensed this panicked feeling in me, which prompted him to tell me to go home and not worry.

In typical Dick Schwartz fashion, he helped me unburden that little boy inside me while I sat in my car on the side of the road. The next day, when I went in to visit my son, I apologized for overreacting the previous night. I found that I was now able to connect with him, not due to excessive empathy, but because I came from a more compassionate place of love, concern, and genuine curiosity for what he was going through. This became compassion informed by empathy. I was able to listen to my son’s experience and share with him that I, too, have felt similar emotions.

I encourage you to explore and practice the differences between compassion (being with) and empathy (being in) as it relates to helping your clients. See if you can sense what compassion feels like internally and how your clients experience your compassionate presence. Then repeat the exercise by using empathy. What does it feel like for you to get emotional when you’re sitting with someone? Does it make you feel uncomfortable? What are the origins of these feelings? Again, I believe parts need both empathy and compassion from us, as well as from our clients’ Self. Being attuned to this balance, however, can be complicated. Try to determine what percentage of empathy (reliving) and compassion (being with) you need to give to your client’s parts so that they can feel heard, seen, and known by you and the Self within them. This balance helps the client feel safe enough to reveal and release the pain they’ve been carrying.

ARE YOU BORN WITH IT, OR IS IT CULTIVATED?

Many, including me, equate the Self with the soul and believe it connects to a higher source (whatever that means to you personally or to your client). IFS espouses the idea that we’re automatically born with this Self and that it doesn’t need to be cultivated or developed (Schwartz & Sweezy, 2020). Dick Schwartz told me that the Self has an agenda—one of love and connection—but

this agenda isn't attached to an outcome. The Self is like software that utilizes the hardware (our developing human body) to express itself, and it has more to work with as we grow and develop.

I experience a difference between the *human* embodiment of Self-energy compared to a *spiritual* experience of the Self. For example, my human Self is singular, and it has a gender and sexual orientation. My spiritual Self, on the other hand, which I experience at various times in my life, connects to the oneness that is all around and within us and doesn't seem to possess a gender or an orientation.

I believe Self-energy is a “state of being” that lives in the mind and utilizes integrated neural networks in the brain to express itself. The Self connects to our internal energy, as well as the external energy found in our environment. I experience the Self as a maximally integrative state in which all is connected and flows together in unison.

When Tania Singer and I were discussing the neuroscience origins of Self-energy, she mentioned to me that she wondered if it was the *absence* of neural network firing that was connected to the state of Self-energy. She noticed that when she scanned the brains of very experienced meditators, their brains were quickly able to turn off and enter a state of no neuronal activation. This is an interesting idea I hope we'll get more clarity on with further research.

One thing I consistently see is that clients with extreme and extensive trauma histories, such as severe neglect or DID, tend to take months to years to be able to access Self-energy. It can take extended lengths of time for protective parts to get to know and trust the Self enough to relax and allow it to emerge naturally. When we work with protective parts that begin to feel seen and known, I believe they allow us to *access* the Self that's already there. I contend that there's no need to cultivate the Self; instead, recognizing and accepting parts helps to *cultivate access to the Self*. I see the Self as the capable, competent leader that's naturally within all of us.

The Self versus Self-Like Parts

During a particularly poignant session with Mia, a client of mine who carries a diagnosis of DID, one of her Self-like parts shared an important perspective with me about the relationship between her Self and her parts. Mia's Self-like part started explaining to me how these parts worked. She said: “I have to tell you, Frank, I know the Self has good intentions, but it scares some of these parts. They feel like the Self is trying to replace them. They're trying so hard to erase the past and never let it surface again. And sometimes, they feel upset with the Self because it seems okay being with all the

bad stuff that happened in the past. It's going to take these parts some time to get used to the Self in a way that doesn't feel threatening to them and their stories. Remember, parts hold most of the real estate in here, and they feel like the Self is invading their turf."

As Mia's therapy continued, her revelations became more surprising: "The Self keeps challenging the parts' views. It makes some of the parts feel like they're wrong for feeling, believing, and responding the way that they do, even though the Self does so in a gentle way. Some of the parts feel threatened by the Self. Last week, for example, the Self expressed another way to handle Jenna when she ghosted us. The suicidal part felt judged and took offense to that comment, and the little one felt shamed. When the Self makes a suggestion like that, some of the parts feel defensive, inadequate, or bad about themselves. Remember, these parts are pretty young. We have to move really slowly to build trust in these relationships. I hope you can understand all of this, Frank."

I told her Self-like part that I understood completely and was grateful for her willingness to share this important perspective with me. "Thank you," I said.

"You're welcome," Mia's part replied. "I felt that it needed to be said."

Hearing this perspective from Mia's Self-like part helped me and my parts relax. It made me refrain from trying so hard to get her parts to acquiesce in an effort to help her Self come forward. Mia's Self-like part gave me a birds-eye view of the fascinating world of our parts. It fostered a deeper respect for and understanding of these parts and their pace.

So how do you differentiate between the Self and Self-like parts? For me, it has to do with energy—a state of being in which I can distinguish between the two. *The Self is energy-expanding, while Self-like parts with agendas tend to be more energy-constricting.* The Self has a calm power associated with it, and it taps into something greater. Self-like parts have useful energy, which understands, does, and says the right thing and helps figure things out. It's smaller and more localized.

THE SELF AND TRAUMA

What happens to Self-energy during a trauma? Does it stay and endure the unspeakable? Does it leave? Do parts jump in and take over? This is a long-standing debate within the IFS community. I've spent a fair amount of time thinking about this question and have discussed it with Dick Schwartz to make sure my thinking aligns with his.

I believe the Self is preserved and protected from trauma, and there is a resultant chasm created between the Self and parts as a result of overwhelming life experiences. Where the Self goes during a trauma is unclear. Some clients describe the Self as leaving the body, while others feel it remains present but out of harm's way. Whatever the case, parts often hold the experience that the Self has left

them behind to endure the painful experience. Luckily, IFS has a way of repairing the relational breach created between the Self and parts as a result of a violation, which we'll discuss in [chapter 9](#).

It's also possible for a fraction of the Self to be split off or exiled in severe trauma. It doesn't get damaged or sustain a burden per se, but a portion of it can be left behind or remain in the past, while the remainder of the Self moves forward in life. During a particularly intense unburdening of mine, I was surprised to encounter a younger aspect of my Self (not a wounded part) that was stuck in the past. It said: "I'm still here. I've always been here. No one ever saw me [meaning my parents]." There was a beautifully poignant reunion or integration between the portion of my Self that remained in the past and my Self of today. I was so surprised by this experience that I quickly called Dick to make sure this was consistent with IFS and his understanding of the Self. He said it was. A portion of the Self can sometimes get exiled during extreme trauma.

I also believe the opposite is true too: *Self-energy, or love, is the antidote to trauma*. Self-energy has the infinite capacity to *be with* and heal trauma. We see this over and over again during the unburdening process. The Self doesn't directly experience, support, or condone abuse within us or others; it overcomes and rises above it. This is one of the great gifts of IFS, the healing capacity of the Self. In other words, in its most basic form, love heals trauma. Love, in fact, conquers trauma.

I have certainly heard parts say things like: "You left me to be raped by Uncle John." Here, parts relay the experience that the Self has left them behind to endure the abuse. I have also heard parts say: "I have to drink. It's my job. It's what I've always known. I have to show up and stop the pain." In this scenario, parts believe it's their role to serve and protect the Self during and after the trauma. I've heard the Self say: "I was up in the corner of the bedroom looking at that sweet little girl being hurt by Uncle John." Here the Self describes leaving during an overwhelming event. I have also heard the Self describe being hijacked by triggered parts: "The anxious part keeps jumping in and taking over" or "Someone just took me out right there."

I believe all of the following are true. *Our essence or our soul is protected from trauma. Parts are left behind, feel abandoned by the Self, and are forced to endure the pain and nightmare of the trauma. And when wounds are triggered, protective parts will jump in and take over the Self in the service of protection.*

Remember, it is common for parts to be angry with the Self for abandoning it during the trauma. These parts often have no real interest in connecting with the Self. Dick Schwartz mentioned to me that clients with DID commonly have parts that *hate* the presence of Self-energy because, historically, perpetrators have attacked it whenever it was present. Hence, when the Self shows up, clients can have parts that get scared and are afraid of perpetrator retribution.

With many trauma clients, we look for what *percentage of the client's Self* is present at any given moment. We know that each time a part feels seen, heard, and validated, it relaxes a bit further and more Self-energy is accessible in their system. Be aware that this can take a long time with your more severely traumatized clients. Their parts have been “running the show” for many years, and they’re used to being in charge and making decisions for the system.

Case in point: June, one of my more dissociative clients, shared a beautiful and clear example of this very dilemma during a therapy session. She said: “I am the Self, and I’m truly sorry that, during the abuse, I left the body behind. It was reflexive, my essence came with me, and the body mass was left behind. There are other times, when the voices get so intense and jump in and take over. They can be really powerful sometimes. It’s hard for me to stay during those times because they’re so strong when they get triggered.”

Anthony, a different client, shared: “Any expression of the Self was dangerous for me as a child. If I were honest and spoke truthfully about what my father did to me, I would have gotten killed. Expressing my love of art and music would have only caused the bullies at school to attack me more. There really wasn’t room for my Self and my parts to coexist back then.”

Another client described her experience of the Self as follows: “It’s impervious. It’s impenetrable. When I’m in alignment, I have mine and they have theirs. It’s clear; it’s the end of the story. On the other hand, when I’m out of alignment, I’m trying to be what other people want me to be, or I’m trying to give them what they need from me. I’m beginning to believe there’s a connection between my being out of alignment and all of my medical issues.”

I certainly know this to be true for me personally. I grew up responding to what other people needed and wanted of me in the service of self-survival. I’m now aware that when my asthma flares up, for instance, it’s a sign that I’m out of alignment, that I’m disconnected from my truth and from what feels right for me. Sometimes it can take me a while to fully listen to the message that this part (that is, the part that expresses itself through my asthma) is trying to communicate with me. But when I can reconnect and freely feel and speak to the truth in my life, my asthma typically resolves.

Pain and trauma are a normal part of our *human* experience here on Earth, and the aspect of us that comes from the source (i.e., the divine) is protected and doesn’t suffer or sustain trauma. Unfortunately, parts of us are left behind to do the heroic job of enduring the trauma, protecting the exiled parts of us that hold the pain, and ultimately preserving the goodness and purity within us that is our Self. In this view, evil does not win over goodness.

THE CONTAINER THEORY

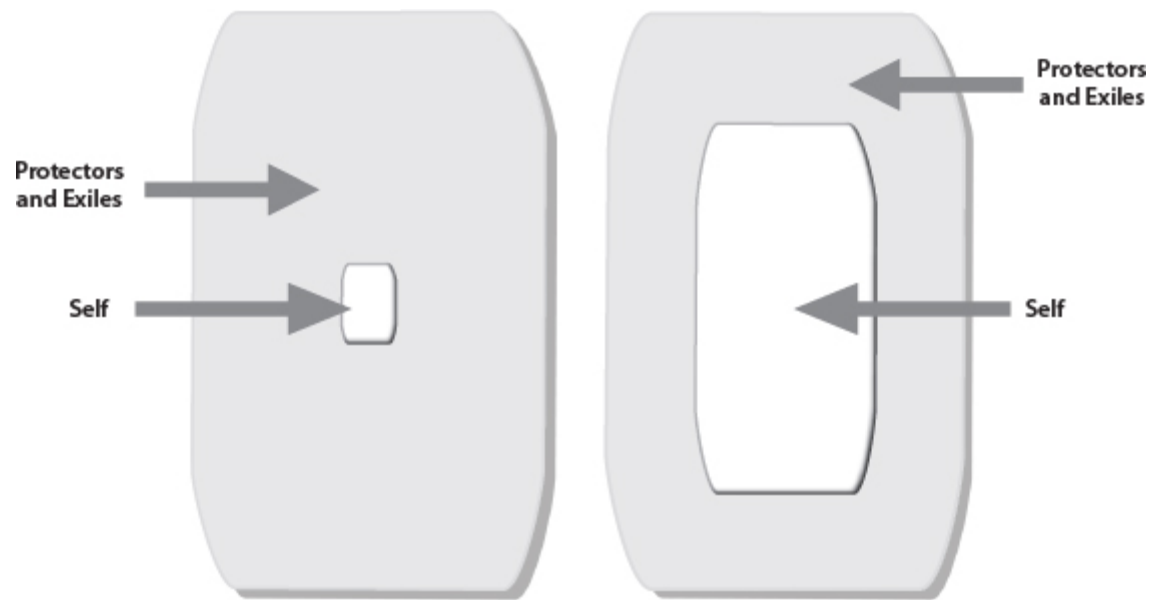
As we've discussed, the Self is preserved during overwhelming experiences, and protective parts are forced to take on extreme roles within the system to protect and fend off the resultant heartache. It's almost as if each person or human body has a *finite amount of space* within itself, and when most of the space inside is taken up by numerous protective parts and several wounded exiles, there is, by default, very little residual space for the Self to occupy.

I call this the *container theory*. How much space do parts take up, and how much space is available for the Self? In severe trauma, the urgency and necessity for protection require that most of the space in the container be consumed by intense and energetically large protective parts—thus leaving little available room for the Self. Let's say that protective parts and exiles collectively occupy 85 percent of the container; that means only 15 percent remains for the Self. With these clients, I intermittently observe the Self emerging. I know it's in there, but there's not a lot of space for it, and it doesn't have the opportunity to show itself very often due to the dominance of space-occupying parts.

However, as we gradually get to know the players in the system, and they begin to trust us and the client's Self more, these hardworking protective parts tend to relax, and the Self emerges more often. In my experience, this is a slow process that requires an enormous amount of patience by even the most skilled therapist.

Again, to what degree does the Self actually leave? To what extent are parts so numerous and active that there's not enough space left for the Self? I don't have definitive answers to these questions. But I'll continue to remain observant and listen to what my clients' parts and their Self tell me about these various scenarios. By staying in the conversation and trusting the process, I'm certain we'll get more clarity over time.

We're all a combination of the Self and parts, and it's the flow between them that brings us into balance and harmony. Trauma disrupts this balance because parts are forced to carry the pain or to work extremely hard to keep the wounds at bay. This burns an enormous amount of energy and occupies lots of space within the container. When we help parts release what they're forced to do or carry, there will be a greater percentage in the container that the Self can occupy.



CHAPTER 8

Steps 5 and 6: BeFriending, Finding the Job, and Finding the Fear

IFS AS INTERNAL ATTACHMENT WORK

Once the target part has separated from the client's Self, and the Self is accessible and connected to the part, it's time to begin fostering an internal relationship between them. Remember that there was a chasm created between them as a result of a relational violation once upon a time. Now we're ready to repair that connection. There is a back-and-forth process that is important and often overlooked by therapists at this stage, as demonstrated in the following hypothetical therapist-client dialogue:

"Nancy, how do you feel toward the part that eats in excess now?" the therapist asks.

"I really care about her and see that she's just a little girl who's been holding so much for so long," Nancy says.

"Send her that caring energy right now, Nancy, and see if she's willing to take it in," the therapist suggests.

Often, therapists identify Self-energy in their clients and feel like they're good to go. But that's only half of the story. We want to make sure that this is a two-way street by asking questions like: "How do you feel toward her (Self-to-part connection)? Is she willing or able to take that in (part-to-Self connection)?" Often, parts aren't willing or ready to accept positive Self-energy. They either don't trust the Self yet or are in a relationship with a Self of the past. (More on this to come in [chapter 9](#).)

Helping clients foster this internal relationship between their target part and the Self is something that takes time, especially in relational trauma, but allowing it to develop slowly over time will pay off in the long run. Be aware that this is new for the part—to have someone who cares for it and genuinely wants to help it. Just as you'd gradually introduce a stray cat into your home as a pet, it's important to proceed gingerly and allow the relationship to deepen over time.

LEARN ABOUT THE POSITIVE INTENTIONS

Once the relationship between the Self and the part feels somewhat secure, we proceed with helping the client learn the part's job or role. Think of this as the beginning of a friendly interview. Ask questions such as: "What's your primary goal for Nancy? How long have you been helping her in this way? How effective do you feel in this role?" It's important for you and the client to learn about the part's intentions from its perspective or point of view. *Once you both genuinely comprehend and acknowledge the positive objectives of a protective part, a natural relationship and compassionate stance toward that part can develop.* Engage in affirming dialogue, like: "Of course, it makes sense that you found eating as a way to keep the depression at bay. I get why you feel the need to do that. Nancy, are you hearing this? How are you feeling toward this part now, after you've heard how it's trying to help?"

Ideally, Nancy would respond in a similar fashion: "I had no idea that the eating part was trying to help me not feel so depressed and lonely. I know my childhood was really awful at times. I'm appreciative of her and really admire her effort."

To which, you'd say: "Let her know that now, Nancy, and see how she responds."

If Nancy says something like, "She is loving that," you know a two-way connection has been established.

Understanding the positive intention of protective parts opens the door to repairing the internal chasm that occurred during the original assault. I'm always looking for that heart-opening experience within myself and within my client. It's an important first step in gaining permission from the part to access the wound. Genuine permission occurs with a heartfelt awareness of the part's true intention. *Learning about the positive intention is learning about its job.* Once the job is revealed, it's a good idea to ask the part what else it wants to share, as feeling heard, known, and seen goes a long way toward getting permission to access the wounds it protects.

FOCUSING ON FEAR: EXILES REVEALED

Once you and your client have aligned with the positive intention of the protective part, it's essential to learn about its fear. Ask the client's part this crucial question: "*What are you afraid would happen if you didn't do this job?*" As many of you have learned from your Level 1 training, this question will either reveal the exile that the protective part is protecting or reveal a different part within the client's system that is polarized from (or in conflict with) the protector on which you're focusing. The client may say: "If I stop drinking, James (another protective part) will get suicidal" or "If I stop drinking, James will see how alone he is in the world (an exile)." Try not to get frustrated if this question doesn't reveal either of these responses. Remember that protective parts are good at being protective; they

don't like to reveal important or vulnerable information so easily. In these circumstances, asking the "And then what?" question is often helpful. This is particularly useful with hypoaroused parts or more blunted systems that tend to respond with one- or two-word answers. Here's a constructive therapist-client dialogue to expound on this point:

"Jay, what is this part afraid will happen if it isn't anxious any longer?"

"That would be unknown," Jay says.

"And what is scary about the unknown?"

"We would lose control," Jay says.

"And what if you lost control?" the therapist asks.

"We don't like feeling out of control."

"What would be the worst-case scenario if you did feel out of control?"

"Oh no, we won't ever do that again."

"So let me see if I get this right: You're fearful of letting go of the anxiety because you're afraid of feeling those same out-of-control emotions you felt as a little boy when you were bullied at school? Is that right, Jay?"

"Yes, exactly," he says.

In this example, using the what-if question reveals the exile (feeling out-of-control emotions during abuse) underneath the protector (the anxious part).

ADDRESSING PROTECTOR FEARS

A common frustration and one of the biggest roadblocks for therapists new to IFS is addressing protector fears. It's common to get stuck, not know what to ask next, give up, and resort to your familiar way of providing therapy. For nearly 35 years, Dick Schwartz has been addressing protector fears with clients. I often watch him at work with amazement, realizing how masterful and efficient he is at convincing protectors to relax.

Addressing protector fears is one of those steps in the IFS model that takes practice, trial-and-error tenacity, and patience. The chart on the next page provides a list of common protector fears, along with some common responses. I've expanded this list from the *IFS Skills Training Manual* (Anderson et al., 2017) because I've become aware—with the help of Fran Booth, an assistant trainer

at the IFS Institute—that there are two important components to addressing protector fears: The first is making sure the part feels heard and understood by validating its job and its fear. The second is offering clients an alternative solution to their problem.

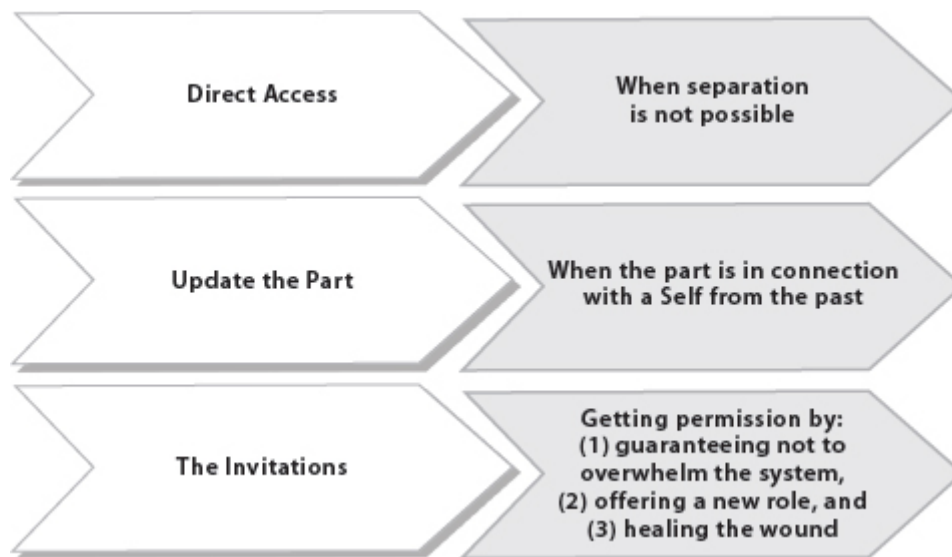
Addressing protector fears is something that takes time and practice. At times, you'll be encouraging your client to have these discussions with their part (which IFS calls *insight*). Other times, when there isn't Self-energy present in your client, you can talk directly to the part (called *direct access*, which we'll discuss in [chapter 9](#)) in the hopes of getting the part to separate and grant permission to proceed.

Protector Fear	Common Responses
"I'm afraid he'll be overwhelmed by the terror." (The most common protector fear in trauma)	• <i>Validate:</i> "I totally get that you're afraid Steve will be overwhelmed by the amount of pain and terror that's in there. That makes sense." • <i>Alternative solution:</i> "What if I were able to guarantee you that the part holding the terror won't overwhelm him? I'm not interested in anyone in there reliving what they went through. There's a different way to do this—by having the parts share a little bit at a time. Are you interested in that?" (See how to deal with overwhelm in chapter 9.)
"If I step back, then what will I do? This is what I've always done."	• <i>Validate:</i> "I hear that you're afraid of losing your job. Is that correct?" • <i>Alternative solution:</i> "We are not interested in getting rid of you in any way. But we can help you no longer feel anxious all the time. What if you were freed up to do something else? Are you interested in that?"
"If I step back, the suicidal part will take over."	• <i>Validate:</i> "I hear that you're afraid that another part will take over if you step back. That's a legitimate fear; it has happened before." • <i>Alternative solution:</i> "I don't want any part to feel the need to take over. We're going to listen to all parts and only move forward when they've all been heard and permit us to proceed, including the suicidal part."
"If you know me and what I've done, you'll hate me."	• <i>Validate:</i> "It sounds like you're afraid that I or Steve's Self might judge you if we get to know you better and learn more about what you needed to do to protect his little boy. Is that correct?" • <i>Alternative solution:</i> "I can assure you that I don't feel any judgment toward you at all. I know that you've tried your best to protect little Steve. If you ever see or feel me judging you in any way, please let me know. Are you willing to do that?"
"If anyone ever finds out what my dad did to me, my family would never talk to me again."	• <i>Validate:</i> "I hear that you're afraid of exposing this secret and that if your family found out, they would reject you. Is that right?" • <i>Alternative solution:</i> "If you permit us to heal the wound, you'll have the choice to tell whomever you want to tell. It will be your decision."
"You can't handle my pain."	• <i>Validate:</i> "I'm hearing that you're worried about my ability to cope with and process what happened to you. Is there anything I've said or done to give you that impression? Or are you just afraid this might be the case?" • <i>Alternative solution:</i> "I've been doing this work for a long time now. If you ever perceive anything that makes you suspicious of my ability to be with your feelings, please let me know. I also want to let you know that we're going to heal the pain so you will no longer need to feel it yourself. How does that sound to you?"
"If I get better, we won't work together anymore."	• <i>Validate:</i> "I hear that you're afraid of losing a connection with me if you get better." • <i>Alternative solution:</i> "You can choose how long you want to stay in treatment with me. I also know that when the wound heals, your younger parts will feel safely connected to you and won't feel the need for my help in the same way that they do right now. Does that make sense to you?"
"Every time things get better or I feel happy, something terrible happens."	• <i>Validate:</i> "I'm hearing that feeling good is scary. Is there anything else you want to share with me, or do I fully grasp your concern?" • <i>Alternative solution:</i> "What if there was a way for you to let go of that anticipatory fear? What if you could trust that Steve would be able to handle whatever came up, both the good and the bad, and you didn't have to be involved in his current life anymore? Are you interested in that option?"
"This Self you keep referring to doesn't exist in me. I'm the exception to the rule. We (Steve's parts) handle everything for him, and we've been doing that his whole life."	• <i>Validate:</i> "I get that you've needed to handle everything for Steve up to this point. I also understand that he hasn't shown up much and that you don't even believe Steve exists in the way that I describe him. Is that correct?" • <i>Alternative solution:</i> "What if I could show you there's a place within that can handle all the pain you went through? Would you be interested in that? It would require all of you to step back for that to happen. All I'm asking for is a few moments. Are you willing to try? I know this is a big ask, and I understand if you're not willing to try it. It's your decision."

CHAPTER 9

Beyond the 6 F's for Trauma

One of the main reasons why treating trauma survivors is so stressful is because of the intensity, persistence, and extreme nature of traumatic symptoms, which IFS refers to as “parts.” As mentioned earlier, the 6 F's are the main steps we move through when encountering protective parts. Our goals are to identify these parts, to gain access to Self-energy, to learn about the parts' jobs and fears, and to reveal who they're protecting. Often, these steps aren't enough to gain full permission to access the wounds that tenacious protective parts are keeping at bay. Additional steps I've found to help gain approval from protective parts associated with trauma follow next. I call these extra steps *Beyond the 6 F's*.



DIRECT ACCESS

Another great IFS contribution to the treatment of complex trauma is *direct access* or when a therapist's Self talks directly to the client's part (Schwartz & Sweezy, 2020). Direct access is an essential skill to master, especially when your client is blended with their part or has little or no access to Self-energy. When traumatized clients are triggered, parts can range from physiologically activated or sympathetically hyperaroused to blunted or parasympathetically hypoaroused.

When these extreme parts take over clients, their physiology is dominated by these intense states, and there is little or no access to the Self in those moments. Their greatest chance to unblend or

separate from these severe states is for the therapist's Self to talk directly to the part that has temporarily taken over. This, of course, is contingent on the therapist's capacity to invoke their Self without being overtaken by one of their own parts in response to the client's intensity.

All therapists can benefit from being proficient at direct access and capable of fluidly moving in and out of this mode as if it were second nature, especially with more severely traumatized clients. Using direct access provides a quick and efficient way to overcome the reluctance and stubborn nature of many protective parts that are unwilling to budge. It certainly takes some getting used to, and it can feel a bit strange when you first incorporate it into your practice.

Direct access is one of the first tools I teach during IFS Level 2 trauma training. Many therapists have heard about it in their Level 1 training, but in my experience, few therapists are truly comfortable with using it routinely in their clinical practice. I encourage everyone who works with complex PTSD clients to regularly incorporate direct access into their repertoire of resources. I am confident it will help move treatment forward and enable your clients to more quickly unblend from their entrenched, persistent, and severe parts.

IMPLICIT VERSUS EXPLICIT DIRECT ACCESS

There are two different types of direct access: implicit and explicit (Schwartz & Sweezy, 2020). *Explicit direct access* is when you directly name and address the part aloud and engage it in a conversation. For example, you might say: "So you're the part of Noah that eats in excess, is that correct?" This is in contrast to *implicit direct access*, where the therapist talks to the client's part(s) in an indirect, general, or passive way without specifically calling them out as a "part." For instance: "I sense that you're feeling angry and upset right now" or "I want to make sure everyone feels safe before we proceed."

I use both implicit and explicit direct access, but I'm a bigger fan of the latter, especially when it comes to working with traumatized parts. They can often take over, dominate the client's system, rapidly switch from one part to another, or feel as if they are the actual client. You might, for example, get a response like: "Why do you call me 'a part of Noah'? I *am* Noah."

When such a scenario occurs, I don't spend too much time trying to encourage the part to separate from the client's Self. I may ask once or twice, but if I'm not successful, I move quickly into direct access. Parts will often prefer to talk to me over the client anyway. From the parts' perspective, I haven't abandoned them during the time of the trauma, and they are often curious to get to know me better—to see for themselves if I'm safe and trustworthy. I tend to utilize implicit direct access earlier on in treatment as I'm getting familiar with the client's system. I talk in general terms to a collective

of parts. For example: “I know there’s a lot of you in there who are hurting right now, and I want to let you know that relief is on the way.”

The main goal of direct access is to help entrenched parts separate from the Self. Remember that they may also be interested in getting to know you better, but be careful not to form individual or special relationships with any parts. This could quickly turn dangerous and be experienced as a reenactment for some parts, as this is a common dilemma in some relational violations that incorporated “special” or “secret” relationships.

I always make a habit of sharing with the whole system what I’ve learned from the part during direct access. If the client doesn’t have DID, they’re usually listening in the background to what their blended part and I are discussing. But clients with DID often are not present or listening, or they don’t want to hear what the part and I are talking about. I’ll usually share with my client what I’ve learned from their part because I don’t want to perpetuate any secrets or forge separate relationships with any of my client’s parts. Complex PTSD often involves secrets and hidden relationships that can be quite damaging, and I’m not interested in contributing to that cycle in any way.

Remember that parts know how to write emails and send voice messages too. So be mindful of who’s communicating what, and make sure you don’t inadvertently participate in a reenactment.

I typically continue engaging the part in a conversation until it believes that I fully get its role, concern, and fear, as well as anything else it wants me to know. The part will likely be much more willing to step back when it feels fully heard, seen, and validated.

Here is an example of therapist-client dialogue involving explicit direct access, which shows the type of information that can be learned by talking directly to the part:

“So you’re the part of Sue that’s anxious, is that correct?” asks the therapist.

“I’m myself, and I’m anxious,” responds Sue’s part.

“Yes, I know. Tell me more about your anxiety.”

“It’s really crippling. Sometimes I can hardly function, and I get stomachaches and have trouble breathing.”

“That sounds really awful. How long has this been going on for you?”

“Ever since the cancer diagnosis last September. It’s unbearable to imagine my kids growing up without their mother.”

“Well, that certainly makes a lot of sense to me. What else do you want to share with me? I want to make sure I get all that you’re holding.”

After the part has shared all that it's carrying and feels 100 percent heard, the therapist invites Sue's Self to join the conversation.

"Can you bring Sue back? I want to make sure she heard all of that."

"I didn't realize how hard my anxiety was working on my kids' and my behalf. I also heard how tired it was."

"Great, Sue. Let the anxious part know that you understand the ways it's trying to help and how tired it is. Let it know that we can help, if it's interested."

"I'm suddenly feeling calmer as you say that."

UPDATE AND APOLOGY

Once separation has occurred between the Self and the part and you begin fostering an internal relationship between them, one of the big challenges in working with relational trauma is ensuring that the target part is in connection with the *current-day* Self. This is the second step in Beyond the 6 F's that, if forgotten or overlooked, can cause frustration and confusion within the therapist because, despite moving through the steps of the IFS protocol, no real progress occurs.

The Self is in connection with the part, which typically happens first, but the part is often stuck in the past at the time of the trauma. It may not be the exact moment of the traumatic event. However, at some point around the time the trauma occurred, there was a split between the part that endured the event and the client's Self. The overwhelming nature of trauma caused this chasm. *The target part is often still in relationship with and has feelings toward the Self of the past.*

Here is an example of therapist-client dialogue in which an attempt to update the part is made:

"Amir, can you send your worried part the love that you're feeling for it right now and see if it's willing to take that in?"

"It doesn't want my love. It's looking away."

Note that anytime a part is unwilling to accept positive regard from the Self, it's a sign that it's in relationship to a Self of the past.

"Amir, can you ask your worried part how old it thinks you are? And just say the first thing that comes to mind. Don't filter your answer."

Often, the client will disregard what it hears at first, believing that it isn't possible that the part still sees the client at that age.

“Wow—I hear my part say age 12. That’s crazy.”

“Let him see that you’re no longer 12, Amir.”

Here, we look for any words that elicit Self-energy, like “your core,” “your inner wisdom,” or “the you that’s not a part.” Be careful not to say “your adult self,” as this can address a high-functioning manager part.

“He can’t believe how old I am. He had no idea I actually grew up.”

“Just give him some time to get to know the openhearted you. Ask him what he needs to be able to trust you now.”

A part is typically not aware that the Self has continued to progress beyond the trauma, and they’re not particularly interested in connecting with the current-day Self much anyway. That’s because a part feels resentful for being left behind to suffer the consequences of the abuse. From the part’s perspective, the Self deserted it.

In more severe forms of trauma, like DID and attachment trauma, parts initially often hate the Self for this abandonment. The Self, in turn, is marginally accessible, inconsistently shows up, and has a hard time reliably committing to the part. Parts are well aware of this and therefore hesitant to connect with the Self of today until the relationship feels safe and dependable. Here, therapists should take the time to cultivate this connection between the current-day Self and the target part because once this relationship is secure, gaining permission to access the wound is much easier.

We assist in repairing the chasm that occurred between the part and the Self of the past by introducing the part to the Self of today and supporting the newfound connection between them. *It’s important for the current-day Self to apologize for this breach (even though it didn’t occur intentionally) and for not being able to protect the part in the way that it needed and wanted back then.*

After the part is introduced to the Self of today, I frequently will ask the client’s Self if there is anything it would like to say to that younger part now. When the Self is fully present, I’ll typically hear things like: “I’m so sorry I wasn’t there for you in the way that you needed me to be. I was 10 years old also, and I did the best I could. I’m truly sorry.” This kind of response is a useful barometer of Self-energy because when there isn’t enough Self-energy around, these apologies can feel hollow and inauthentic. When the Self is present, it’s heartfelt.

Case in point: An adult client named Maya was raped by a group of teenage boys when she was 8 years old. She described how her parts felt the need to come in and rescue her younger Self:

“She couldn’t protect herself, so we had to jump in and save her,” one of Maya’s protective parts said during a session.

Maya’s current-day Self responded: “My parts don’t trust me. I let them down back then. I couldn’t handle what happened to us, and I really feel sorry about that now. I remember feeling defeated and all alone. Nobody in my family ever asked me what happened that day. They knew something bad went down, but no one cared enough to ask me anything about it.”

Maya described how her protective parts carry the burden of believing they needed to protect the Self back then and how her current-day Self feels sorry about not being strong enough to endure the abuse. Her protective parts bear the burden of the trauma, and her Self holds the regret about not being able to withstand the rape and the grief associated with abandoning her younger parts. Repairing this relationship is an important aspect of the healing process, and it often *happens slowly over time, requires mutual trust, and needs to be handled with care.*

GRATITUDE AND REGRET

As we work with extreme or proactive protective parts, they will eventually begin to feel heard, known, and seen in a way that allows them to comfortably begin to establish a newfound relationship with the client’s current-day Self. As this Self-to-part relationship develops, common feelings will arise, including gratitude and regret. This newly formed relationship allows the Self to share its gratitude for how hard the part was trying to help and for the gifts that came along with its job.

For example, a client named Joseph said: “I really appreciate my résumé part. He brought so many wonderful opportunities to my life and helped me achieve excellence in my career.”

His part, in turn, was able to receive this gratitude and, after many years, finally felt truly appreciated. In response, Joseph said: “He is smiling when I say that to him. He loves the recognition.”

Equally as important, and often more poignant, is when the Self also compassionately shares in the sorrow and regret with the part, with remarks like: “I really get why you’re feeling so bad. Going too broadly at work in the service of achievement, and not being able to dive deeply into your true passions, kept you from feeling joy and creativity. I also know that you may have lost out on deeper connections in life because of always working so hard to achieve. I’m sad for you too.”

It’s powerful to share this space with your client when both the Self and the part can join together in gratitude and regret in a heartfelt, compassionate, and non-shaming way over the gifts and shortfalls of the protective part’s job. When this degree of intimacy develops between them, it suggests that the time is right to safely move forward toward the underlying wound.

GETTING PERMISSION

Once the Self of today is in a relationship with the target part, and that part's job and fear have been identified, the one remaining goal is to gain the part's permission to access the wound. Protectors often grant permission once they are given an offer they believe they can't refuse. Remember, these parts have been doing this job for a long time, and their attempt at protection has been deemed marginally effective at best.

The first offer often presented to protective parts occurs when a part begins to see, feel, and trust the presence of Self-energy. This provides the part a new and different option, the possibility of lightening their load, not having to work so hard, and letting down their guard for once. The part begins to experience the strength, power, and capacity of the Self. In turn, the part allows itself the luxury of slowing down the intensity and frequency of its load. It knows that its job is still necessary, as the wound is still present inside. But for the first time, help has arrived, and there's the option of some relief.

This is an interim solution to its problem; the part becomes aware that only when the wound is fully healed can it truly be freed from its protective role. Yet the respite feels good to an exhausted and overworked part. *This offer allows for a decrease in the constancy of its job because the part has the support and competence of Self-energy.*

As is the case with explicit versus implicit direct access, I'm also a fan of what I like to call *explicit permission* compared to *implicit permission*. I prefer to hear parts explicitly say: "Yes, then I'll give you permission. I'm nervous but I'm going to do it." Often, when parts feel validated, they'll spontaneously soften or step back (i.e., grant implicit permission). With implicit permission, though, you'll often notice that parts will jump back in when they get scared or overwhelmed by feelings.

Therefore, it's important to invest time up front in getting parts to genuinely feel heard, seen, and appreciated by the client's Self, as well as by the therapist, and for these parts to feel comfortable in giving explicit permission to proceed. When they grant full permission, protective parts rarely jump in as the unburdening process unfolds. There is also little, if any, backlash after healing happens, and the overall process usually takes much less time.

THE INVITATIONS

Once parts trust, settle into, and benefit from the offer of the Self's presence, it's time to provide them with one of three invitations:

1. *Deal with the overwhelm.* For example, offer the following: “What if we were able to guarantee that the little girl (an exile) or the suicidal part (an extreme protector) would promise not to overwhelm the system with their intense feelings? Would you be interested in that?”
2. *Offer a new role.* For example, suggest the following: “What if you no longer had to be anxious or depressed or eat in excess? Would you be interested in that? What would you rather do instead?”
3. *Offer to heal the wound.* For instance, recommend the following: “What if we were able to heal little Billy and you no longer needed to protect him? Might you be interested in that?”

These three invitations are quite compelling to protective parts. They offer the possibility that the part can be relieved of its duties. Dick Schwartz has said that an invitation “offers hope to a hopeless system.”

DEALING WITH OVERWHELM

As previously mentioned, addressing protector fears can be one of the more challenging therapist tasks and the cause of frustration and a loss of faith in the IFS model. With most trauma, and for complex PTSD in particular, coping with overwhelm is the most common protector fear to be addressed.

I’ve often heard Dick Schwartz say: “Once a part agrees not to overwhelm the system, it won’t. It never goes back on its word.” However, for a long time, this wasn’t the case for me in my practice. I would ask client parts to not overwhelm, and they would agree but would consequently “storm the gate” by completely taking over my client when things started getting intense. I would ask myself: “What was I doing wrong?”

Over time, I’ve learned a few things about addressing overwhelm. First, you need to fully believe that parts are capable of not overwhelming the system and to embody this from a place of Self-energy before you can suggest it to the client’s part with confidence. Now I say to parts: “I know you can do this. I’m sure of it. I don’t think we should proceed unless you promise not to overwhelm Anthony. From my experience, nothing therapeutic happens when you relive your traumatic experiences. I want you and everyone else in there to be safe.”

When parts continue to demonstrate apprehension, I’ll say: “Would you like to practice this with me? I want you to share all that you’re holding for Anthony. But I’m asking you to share it a little at a time or in manageable chunks. This way, Anthony will be able to stay present and be there for you in

a way you haven't experienced before." Here, I'm using direct access with the client's overwhelmed part. I may even suggest that we perform a little experiment together and have the part share 1 or 2 percent of its feelings so that the part can directly experience its capacity not to overwhelm.

Now, I can say with confidence and experience that once a part promises not to overwhelm, it won't.

A NEW JOB AND HEALING THE WOUND

Dick often says: "There are three things I continually offer a protective part. First, I give it control by saying, 'You're the boss. We aren't going to do anything without your permission.' Second, I validate its experience with words like 'I get why you are feeling this way' or 'That makes sense.' And third, I offer the protective part hope by saying, 'What if we were able to heal the wound?'"

Remember that protective parts had to do their job because it was the solution they devised when they were all alone and trying to manage overwhelming life experiences. They try to solve problems rooted in relational boundary crossings, power imbalances, shame, and humiliation. Protective parts pull from what they know and have internalized from their environment, and they are often young—with limited developmental and cognitive abilities.

Our job is to give protective parts back the control that was taken away from them, to validate their valiant efforts, and to then offer them an alternative solution to their problems. Sometimes I feel more like a salesperson than a therapist in this endeavor. Most of the time that I spend with protective parts involves selling them on the idea that, if they give us (the client's Self and me) space and permission, we can help them release the wounds they're so steadfastly protecting while simultaneously offering them a new role in the system. You may have to present this offer several times, sometimes even hundreds of times, to zealous protective parts before they establish enough trust in you and your client and are willing to step back. Clearly the solution they originally settled on isn't working so well or the client wouldn't be seeking therapy. But we don't need to point out the ineffectiveness of the protective part's job—it's already aware of this on some level. Instead, we can offer it a novel solution to its dilemma, one that the part hasn't thought of yet. We can invite it to unload the burden of its job and enjoy the freedom to move forward.

The following therapist-client dialogue underscores how this offer can work:

"What would I rather do? I've never even thought about that before. This has always been my role. This is all I've ever known," the client says.

“I’d like you to take a moment and just think about the possibility of being able to do something differently.”

The client asks: “What would I do then?”

“Whatever you want to do. It’s totally up to you. It’s your choice.”

“I’m going to need to sleep on that for a while. No one has ever offered me anything like that before.”

“I’m offering this right now, and it’s a real possibility for you.”

The idea of being freed up to do something completely different is extremely enticing to protective parts. Their experience is that they *have* to do their job. They are like hamsters racing endlessly on a wheel, stuck in this repetitive role. And they’re correct: They *do* have to continue performing this job until the wound heals. This is why I never spend too much time trying to convince parts to, for example, stop drinking or binging or obsessing. They feel like it’s their duty. I respect their view but offer them a different option. Fortunately, they want help; they just haven’t been able to find an alternative solution by themselves. The therapist’s role is to provide hope.

Being respectful and getting permission are my main priorities when encountering traumatized protective parts. My job is to help them feel valued and appreciated for their heroic efforts, aid them in relinquishing their duties, and get them to allow us to unload the damage caused by other people’s harsh words, poor behavior, and lack of caring.

WORKING WITH PROTECTIVE PARTS IN TRAUMA

For best results, follow these steps:

1. Identify the client's target part.
2. Get the client to focus their attention internally.
3. Begin to explore the part with the client, and ask the part for some space (separation or unblending).
4. If the part is not willing to unblend, talk directly to the part (via direct access).
5. Once separation has occurred, identify Self-energy.
6. Foster the Self-to-part relationship.
7. If the part isn't interested in connecting with the Self, update the part to the current-day Self and have the Self apologize for its lack of support during the trauma.
8. Help the Self learn about the part's positive intentions, including its job and fears.
9. Address the part's fears, and help the part guarantee not to overwhelm the system.
10. Offer the part a new role, and get permission to heal its wounds.